COMPREHENSIVE PROJECT PROPOSAL & OPERATIONAL ROADMAP

Project Title: Establishment of a 450-Bed Free/Subsidized Dialysis Hospital, Ambulance Fleet, and Wheelchair Service Unit
Target Beneficiaries: 2,500 Low-Income Renal Care Patients
Entity Structure: Section 8 Non-Profit Healthcare Company (India)

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4. CHUNK 3: HOSPITAL OPERATIONS, EQUIPMENT SPECS & PHASED EXECUTION ROADMAP

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1. EXECUTIVE SUMMARY & PROJECT VISION

1.1 Executive Summary

End-Stage Renal Disease (ESRD) poses a catastrophic economic burden on Below Poverty Line (BPL) and low-income families in India. A chronic dialysis patient requires 2 to 3 treatment sessions per week (120 to 150 sessions annually) to survive. The standard market cost per session in private facilities ranges from ₹1,500 to ₹2,500, excluding erythropoietin injections, diagnostic tests, and transport—making treatment unaffordable for millions.

This project establishes a dedicated, self-owned **450-bed charitable dialysis hospital** supported by a **30-vehicle specialized ambulance fleet** (including wheelchair-accessible transport vans) to deliver **free and heavily subsidized life-saving renal care to 2,500 registered patients**.

1.2 Model Transition: Outsourced vs. Self-Owned Hospital

Initial strategic projections evaluated outsourcing 2,500 patients to third-party commercial dialysis centers. Comparative financial analysis proved that establishing a **self-owned**

charitable healthcare infrastructure reduces annual operating expenditure (OpEx) by over 60%, ensuring long-term fiscal sustainability.

Financial Parameter	Outsourced Healthcare Model	Self-Owned Charitable Facility Model
Average Cost per Dialysis Session	₹2,200 – ₹2,500	₹800 – ₹950 (At-cost consumables & utilities)
Annual Dialysis OpEx (2,500 Patients)	~₹81.9 Crore / year	~₹28.0 Crore / year
Ambulance & Transport OpEx	~₹6.0 Crore / year	~₹3.2 Crore / year
Total Annual Operational Budget	**~₹87.9 Crore / year**	**~₹31.2 Crore / year**
Operational Savings	Baseline	**~₹56.7 Crore Saved Annually (64.5% Cost Reduction)**

1.3 Core Project Impact Objectives

- **Direct Dialysis Capacity:** Perform up to **300,000 to 375,000 dialysis sessions annually** across 3 operational daily shifts.
- **Mobility Support:** Provide free, scheduled door-to-door wheelchair ambulance transport for 2,500 mobility-impaired patients.
- **Financial Protection:** Prevent medical debt-induced poverty for 2,500 families by covering 100% of treatment, consumable, and transport costs.

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2. CHUNK 1: LEGAL ENTITY INCORPORATION, TAX COMPLIANCE & ZERO-COST SETUP

2.1 Entity Selection: Section 8 Non-Profit Company

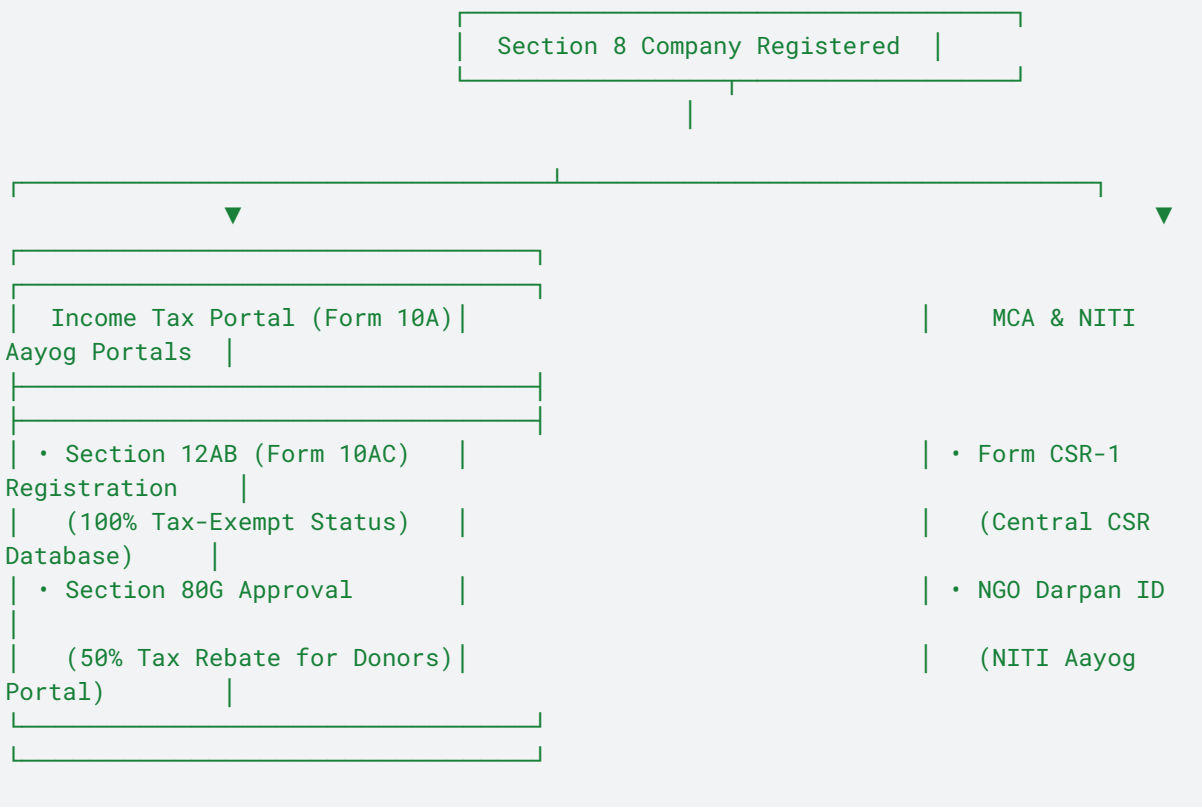
The organization will be incorporated as a **Section 8 Company** under the Companies Act, 2013 (Ministry of Corporate Affairs). Section 8 status is selected over traditional trusts due to higher corporate governance standards, strict digital reporting, and preference by corporate CSR boards.

Key Pre-Requisites

- **Minimum Board:** 2 Directors (including Founder).
- **Authorized Capital:** Fixed at **₹1,000,000 (₹10 Lakhs)**—kept below the ₹15 Lakh threshold so the Ministry of Corporate Affairs (MCA) waives standard incorporation fees (₹0 MCA Fee).
- **Statutory Attestation:** Form **INC-14** certified by a practicing Chartered Accountant (CA), Company Secretary (CS), or Advocate.

2.2 Tax Registrations & Exemptions Workflow

None



1. **Section 12AB Registration (Form 10A):** Grants **100% Income Tax exemption** on all voluntary contributions, grants, and corpus investment yields received by the non-profit. Issued electronically as Form 10AC (Valid for 3 years provisionally, followed by 5-year renewals).
2. **Section 80G Approval (Form 10A):** Enables donors (individuals and corporations) to claim a **50% deduction on their taxable income** for contributions made to the organization.

3. **Form CSR-1 (MCA Portal):** Mandatory registration with the Ministry of Corporate Affairs to legally accept Corporate Social Responsibility (CSR) funding from public and private companies.
4. **NGO Darpan Registration (NITI Aayog):** Assigns a unique ID required to partner with government bodies and apply for public land or health grants.

2.3 Regulatory Compliance & Section 11(5) / Section 13 Rules

- **85% Spending Mandate (Section 11(1)(a)):** The entity must apply at least **85% of its annual income** toward charitable medical operations, hospital OpEx, or direct patient care within the financial year.
- **15% Accumulation Rule:** Up to **15% of annual income** can be set aside tax-free into a long-term capital reserve or endowment fund.
- **Approved Investment Modes (Section 11(5) & Rule 17C):** Endowment capital must be held strictly in designated safe avenues:
    - Public sector bank fixed deposits (FDs).
    - Government securities (G-Secs) and State Development Loans (SDLs).
    - SEBI-registered Shariah-compliant Mutual Funds (e.g., Tata Ethical Fund) or approved PSU bonds.
- **Founder Remuneration Rules (Section 13(2)(c)):** Remuneration paid to the Founder/Managing Trustee is legal provided it is for actual administrative duties (e.g., Executive Director/CEO) and is benchmarked against market rates (~1.0% to 1.5% of total budget). High clinical pay for Chief Nephrologists is valid based on medical credentials.

2.4 Chunk 1 Cost Breakdown & Out-of-Pocket Execution

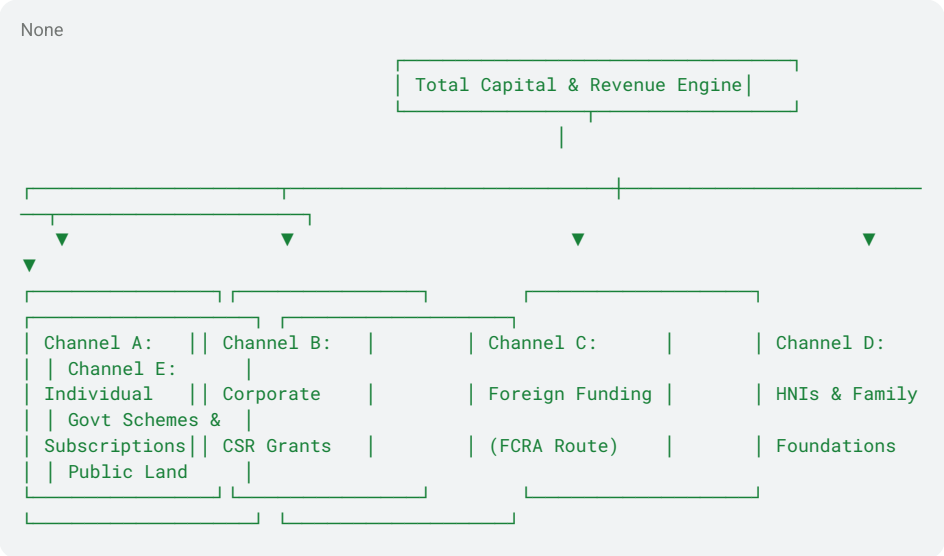
Item / Registration	Government Fee	Professional / Agency Fee	Total Estimated Cost
Name Approval (SPICe+ Part A)	₹1,000	Included	₹1,000
Incorporation Filing (SPICe+ Part B)	**₹0** (Waived under ₹15L capital)	₹2,000 – ₹3,500	₹2,000 – ₹3,500
Class-3 Digital Signature (DSC)	₹0 (Existing DSC used)	₹0	**₹0**
PAN & TAN Allotment	₹131	Included	₹131
Section 12AB & 80G (Form 10A)	**₹0** (Free on Tax Portal)	₹1,500 – ₹3,000	₹1,500 – ₹3,000
Form CSR-1 & NGO Darpan ID	₹50	Included	₹50

Item / Registration	Government Fee	Professional / Agency Fee	Total Estimated Cost
TOTAL INITIAL LAUNCH COST	**₹1,181**	**₹3,500 – ₹6,500**	**~₹4,681 – ₹7,681**

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3. CHUNK 2: COMPREHENSIVE MULTI-SOURCE FUNDRAISING ARCHITECTURE

To establish and maintain a project of this scale without relying on founder capital, funding is structured across 5 distinct, parallel channels.



CHANNEL A: Indian Individual Micro-Donations & Subscription Engine

1. Recurring Monthly Subscription Model

Targeting middle-class and salaried individuals to build a predictable, recurring monthly income stream for operational consumables (dialyzers, blood tubing, chemicals).

- **Target Target Audience:** 5,000 individual monthly recurring donors.

- **Subscription Price Points:** ₹500 / month $(6)|₹1,000/month(12)$ | ₹2,500 / month ($30).

$$\text{5,000 Donors} \times ₹1,000/\text{month} \times 12 \text{months} = \mathbf{₹6,000,000/year (₹6.00 Crore / year)}$$

2. Medical Crowdfunding Platforms

Partnering with platforms (Milaap, Ketto, ImpactGuru) for verified patient sponsorship campaigns (*"Sponsor 12 Dialysis Sessions for an ESRD Patient"*).

- **Tax Lever:** Every individual donor receives an automated Section 80G tax certificate reducing their taxable income by 50%.

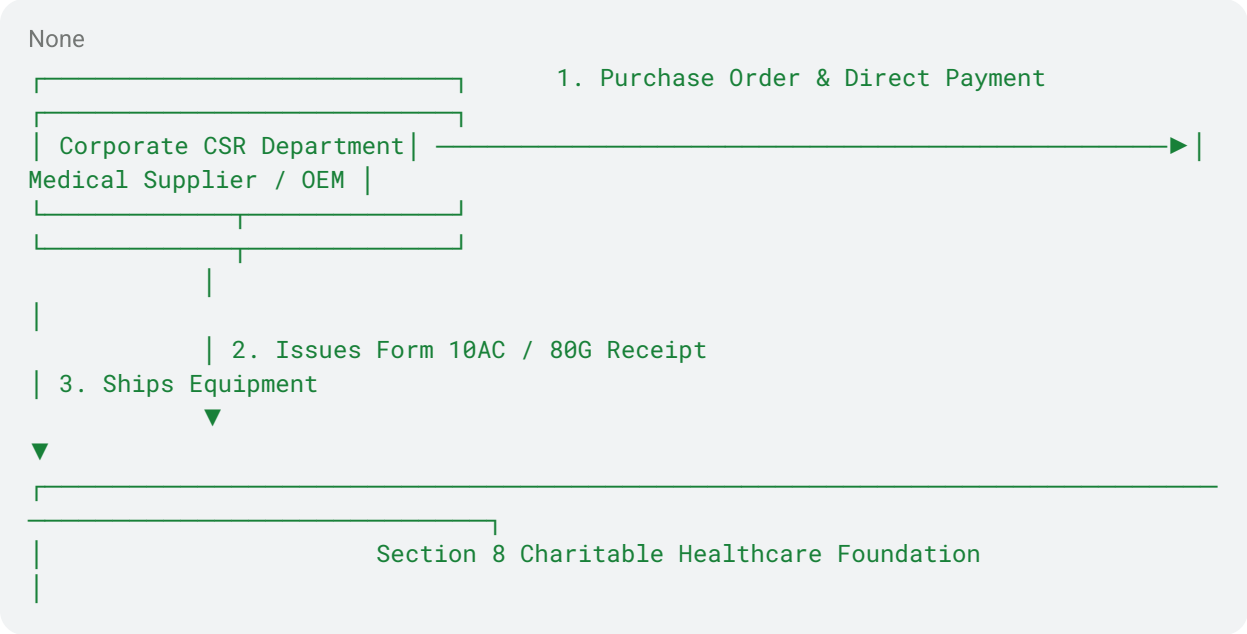
CHANNEL B: Indian Corporate CSR Grants (Section 135 Mandate)

Under Indian law, companies with a net worth of ₹500 Cr+, turnover of ₹1,000 Cr+, or net profit of ₹5 Cr+ must allocate **2% of average net profits to CSR**.

1. "Direct Asset Procurement" Pitching Model

Corporations prefer funding tangible capital assets over transferring cash to early-stage non-profits.

- **Operational Mechanism:** Corporate CSR boards purchase equipment directly from vendors (e.g., Fresenius, Nipro, Force Motors) and transfer the physical assets to the non-profit.



2. CSR Grant Target Allocation

Asset Category	Target Corporate Sponsor Type	Expected Grant Size
Dialysis Machines (Batch of 10–20)	Pharma, Banking, IT Majors (e.g., TCS, Infosys, HDFC)	₹60 Lakhs – ₹1.20 Crore
Industrial RO Water Treatment Plant	Infrastructure, Oil & Gas (e.g., ONGC, IOCL, Reliance)	₹1.50 Crore – ₹2.50 Crore
Wheelchair Ambulance Fleet (5 Units)	Automotive OEMs (e.g., Tata Motors, Mahindra, Force)	₹75 Lakhs – ₹1.00 Crore

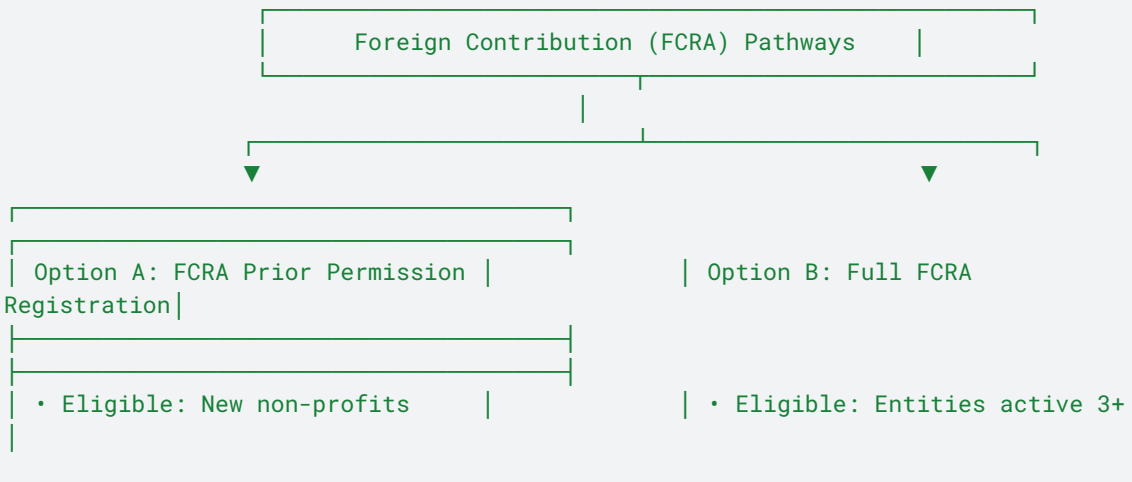
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CHANNEL C: Foreign Donations & FCRA Architecture

Foreign contributions are governed strictly by the **Foreign Contribution (Regulation) Act (FCRA), 2010** administered by the Ministry of Home Affairs (MHA).

1. FCRA Prior Permission vs. Full FCRA Registration

None



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|   (< 3 years old)                |              |   years
|
| • Requirement: Specific donor,   |              | • Requirement: Spent ≥ ₹15L on
|
|   specific project & fixed sum   |              |   charity over past 3 years
|
| • Process: File Form FC-3B       |              | • Process: File Form FC-3A
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- **Phase 1 Strategy (Years 1–3): FCRA Prior Permission (Form FC-3B)**
    - Allows a young non-profit to accept a specific foreign grant from a named international donor (e.g., a foreign foundation or NRI association) for a targeted project (e.g., procuring 20 dialysis machines) before reaching the 3-year operating mark.
- **Phase 2 Strategy (Year 3+): Full FCRA Registration (Form FC-3A)**
    - Grants unrestricted approval to receive foreign donations from any international source for 5 years.

2. Mandatory FCRA Banking Infrastructure

- **Main Designated Account:** Must be opened strictly at the **State Bank of India (SBI), Main Branch, New Delhi (NDMB)**. All foreign funds must enter India exclusively through this account.
- **Secondary Utilization Accounts:** Opened in local operational bank branches to transfer funds from the SBI New Delhi account solely for hospital expenditure.

CHANNEL D: High-Net-Worth Individuals (HNIs) & Family Foundations

1. Naming Rights & Memorial Endowments

Targeting wealthy individuals, industrialist family trusts, and non-resident Indian (NRI) philanthropists through major gift solicitations.

- **Building Naming Rights:** ₹15.0 Crore – ₹25.0 Crore grant.
- **Dialysis Wing Naming Rights (100 Beds):** ₹5.0 Crore grant.
- **Ambulance Bay Sponsorship:** ₹1.5 Crore grant.
- **Individual Dialysis Unit Memorial Plaque:** ₹10 Lakhs per machine.

2. Shariah-Compliant Zakat & Sadaqah Foundations

Establish a dedicated, audited **Zakat Fund Management System**.

- **Clinical Protocol:** Zakat funds are applied exclusively to cover treatment consumables for verified 100% destitute and BPL patients meeting institutional criteria.
- **Separate Accounting:** Zakat receipts are maintained in a dedicated bank sub-account with annual compliance certificates issued to donors.

CHANNEL E: Government Health Schemes & Concessional Land Strategy

1. Scheme Integration (PMNDP & PM-JAY)

- **Pradhan Mantri National Dialysis Programme (PMNDP):** The foundation partners with state health missions to operate as a service provider under PMNDP, securing state reimbursements for BPL patient treatments.
- **Ayushman Bharat (PM-JAY Empanelment):** Empanelling the 450-bed hospital under PM-JAY allows the trust to claim fixed package rates per dialysis session directly from the National Health Authority (NHA).

2. Concessional Institutional Land Allotment

- **Government Policy:** State Industrial Development Corporations (e.g., KINFRA, MIDC, KIADB) and Urban Development Authorities reserve public land plots for non-profit health facilities.
- **Execution:** Apply under charitable quota for **99-year nominal leases at concessional rates (up to 75% below market land rates)** or token rentals (e.g., ₹1/year).

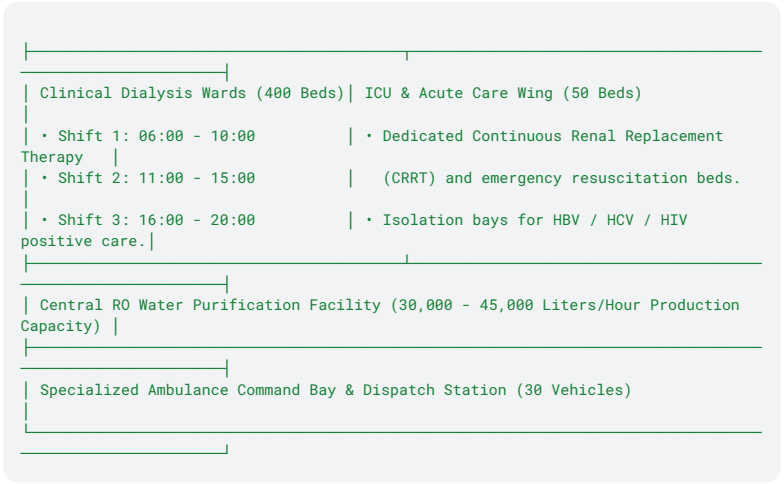
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4. CHUNK 3: HOSPITAL OPERATIONS, EQUIPMENT SPECS & PHASED EXECUTION ROADMAP

4.1 Facility Infrastructure & Layout Specifications

None





4.2 Medical Equipment Specifications & Procurement CapEx

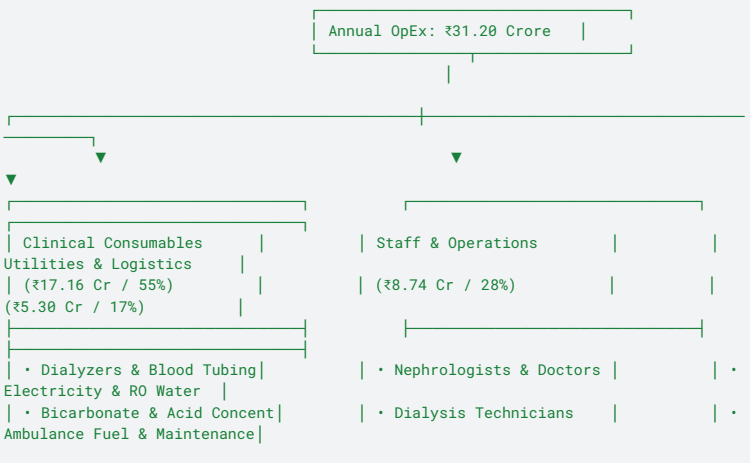
Equipment Item	Technical Specifications	Unit Quantity	Estimated Unit Cost	Total Capital Cost
Hemodialysis Machines	Advanced touchscreen controls, endotoxin filter, ultrafiltration control (e.g., Fresenius 4008S / Nipro Surgial)	450 Units	₹6.50 Lakhs	₹29.25 Crore
Central Industrial RO Water Plant	Double-pass Reverse Osmosis system with UV sterilization, endotoxin filters, SS316L loop pipeline (35,000 LPH)	1 Plant	₹2.50 Crore	₹2.50 Crore
Wheelchair Ambulance Vans	Force Traveler / Tata Winger custom builds with hydraulic wheelchair lift, oxygen delivery, and basic life support	20 Units	₹22.00 Lakhs	₹4.40 Crore

Equipment Item	Technical Specifications	Unit Quantity	Estimated Unit Cost	Total Capital Cost
Advanced Life Support (ALS) Ambulances	Cardiac monitor, portable ventilator, suction pump, defibrillator, stretcher	10 Units	₹32.00 Lakhs	₹3.20 Crore
Dialyzer Reprocessing Systems	Automated multi-station dialyzer reprocessor for safe, hygienic dialyzer reuse tracking	15 Units	₹4.50 Lakhs	₹0.67 Crore
Backup Power Systems	500 kVA Silent Diesel Generator sets + online industrial UPS array for uninterrupted dialysis runs	3 Units	₹45.00 Lakhs	₹1.35 Crore
TOTAL EQUIPMENT CAPEX				**₹41.37 Crore**

4.3 Operational Cost Breakdown (Annual OpEx — ₹31.20 Crore Model)

$$\text{Annual Budget} = \text{Clinical Consumables (55\%)} + \text{Staff Salaries (28\%)} + \text{Utilities \& Fleet (17\%)}$$

None



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| • Heparin & Erythropoietin|          | • ICU Nurses & EMTs       |          | •
Facility Overhead & Waste|
| • AV Fistula Needles      |          | • Administrative CEO/Founder|        |
Bio-Disposal            |
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4.4 Phased Implementation Roadmap

None

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Phase 1: Legal Setup & Seed (M1 - M6)
├── Form Section 8 Entity & Secure 12AB / 80G Exemptions
├── Register Form CSR-1 & Obtain NGO Darpan ID
└── Launch Zero-CapEx Patient Referral Helpline & Government Scheme Integration

Phase 2: Pilot Facility & Fleet Launch (M7 - M18)
├── Acquire Leased 30-Bed Pilot Facility & Procure 30 Dialysis Units via CSR
├── Deploy Initial 5 Wheelchair Ambulances
└── Establish Recurring Donor Network (Target: 2,000 Subscriptions)

Phase 3: Land Allotment & Main Construction (M19 - M36)
├── Secure Concessional Land Allotment from State Authorities
├── Construct Main 450-Bed Hospital Facility & Install Central 35,000 LPH RO
System
└── Secure FCRA Registration / Prior Permission for International Grants

Phase 4: Full Facility Commissioning (M37+)
├── Commission Full 450-Bed Dialysis Ward across 3 Daily Shifts
├── Scale Ambulance Fleet to 30 Vehicles
└── Achieve Full Capacity Serving 2,500 Registered Patients (~300,000
Sessions/Year)
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5. DOCUMENT AUTHORIZATION & SIGN-OFF

Prepared By: Founder & Executive Board
Entity Status: Section 8 Non-Profit Health Foundation (Under Incorporation)
Target Execution Date: 2026–2027 Financial Year

This project document is formatted for conversion into formal submission packages for Corporate CSR Boards, Government Land Allocation Committees, and Income Tax Registration Authorities.